Supplementary Table 1: Study characteristics of included studies.

Author year	Location	Comparison	Participants analyzed	Settings	Baseline prevalence	Coverage (%)	Frequency and duration	Follow–up (months)	Outcome*	Quality rating†
Single dose of MDA										
High baseline prevalence										
Broman[16]	Kongwa, Tanzania	Longitudinal	<8 years	1 village	TF 77%; *Ct* infection: 68.30%	>80	Single	0, 2, 6, 12, 18	TF 57.20%; *Ct* infection:19.20%	2
West[17]	Kongwa, Tanzania	Longitudinal	<15 years	1 village	TF 77%; *Ct* infection: 57%	86	Single	0, 2, 6, 12, 18	TF NR; *Ct* infection: 12%	2
West[18]	Kongwa, Tanzania	RCT insecticide spraying+MDA *vs*. MDA	<8 years	16 neighborhoods	TF 60–70%; *Ct* infection: 29–35%	>80	Single	6,12	TF 43% *vs*. 44%, $P = 0.900$; *Ct* infection 9% *vs*. 7%, $P = 0.450$	1
Laming[19]	Aboriginal community in Australia	Longitudinal	6 months–15 years	1 community	TF 42–55%	57.40	Single	0, 6–8, 12, 24	TF 34%	2
Morberg[20]	Gurage, Ethiopia	Longitudinal	1–5 years	21 villages	*Ct* infection: 56%	Unknown	Single	0, 2, 6	*Ct* infection at 2 months: 8.90% at 6 months: 10.50%	2
Melese[2]	Gurage, Ethiopia	Longitudinal	1–5 years	24 villages	*Ct* infection: 56%	91.90	Single	0, 2, 6	*Ct* infection: 11%	2
Lakew[21]	Gurage, Ethiopia	Longitudinal	1–5 years	40 villages	*Ct* infection: 48.90%	90.60	Single	0, 2, 6	*Ct* infection: 7.90%	2
Chidambaram[22]	Gurage, Ethiopia	Longitudinal	1–5 years	33 villages	*Ct* infection: 43.50%	86–96	Single	0, 2, 6, 12, 18, 24	Treated At 2 months, *Ct* infection: 5.10%	2

									At 12 months, Ct infection: 6.70% At 24 months, *Ct* infection: 11% Untreated: At 12 months, *Ct* infection 17.20%	
Solomon[23] (I)	Rombo, Tanzania	Longitudinal	<15 years	1 community	TF 36%; *Ct* infection: 9.50%	97.80	Single	0, 2, 6, 12, 18, 24	TF 16.30%; *Ct* infection: 0.10%	2
Solomon(II)[24]	Rombo, Tanzania	Longitudinal	1–9 years	1 community	TF 16.30% *Ct* infection: 0.10%	93.10	Single	42, 60	TF 2.60%; *Ct* infection: 0%	2
Chidambaram[25]	Gurage, Ethiopia	Cross sectional	1–5 years	16 peasant associations	*Ct* infection: 35%	91.30	Single	6	*Ct* infection: 7.60%	4
Moderate baseline prevalence										
Cajas–Monson[26]	Kongwa, Tanzania	Longitudinal	<9 years	4 villages	TF 27.80%; *Ct* infection: 23.70%	95%	Single	6	TF NR; *Ct* infection:10.40%	2
Schémann[27]	Segou and Mopti, Mali	Longitudinal cohort (all age) *vs*. (<11 years+women) *vs*. (households of children with TF)	<11 years	3 villages	TF 20.20–23.70%	About 80%	Single	0,1,6,12	TF 6.40% *vs*. 6.80% *vs*. 8.50%	2
Last[28]	Guinea Bissau	Cross sectional	1–9 years	38 villages	TF 22%; *Ct* infection: 26%	70%	Single	12	TF 7.40%; *Ct* infection: 6.60%	4
Harding-Esch[29]	Gambia and	Cross sectional	0–9 years	18 villages	TF 14.90–23.90%	82.00	Single	12	TF 8.00–17.70% *Ct* infection: 3.60–3.80%	4

	Senegal				*Ct* infection 1.80–3.00%					
West[30]	Kongwa, Tanzania	Longitudinal	<10 years	4 communities	TF 1.20–33.00%; *Ct* infection: 5.90–29.10%	>90%	Single	0, 6	TF NR; *Ct* infection: 1.20%	2
Holm[31]	Geta and Sripur wards, Nepal	RCT (Children 1–10 years) *vs*. (households of children with TF)	1–10 years	2 villages	TF10–20%	79%	Single	6	TF 9% *vs*. 10%; $P = 0.330$	1
Burton[32]	Saloum, Gambia	Longitudinal	1–10 years	14 villages	TF 15.40% *Ct* infection: 9.70%	83%	Single	0, 2, 6, 12, 17, 30, 60	TF 4.80% *Ct* infection: 0.60%	2
Low baseline prevalence										
Yayemain[33]	Northern and Upper West Regions, Ghana	Cross sectional	1–9 years	18 endemic districts	TF 6–12%	68.60–93.10	Single	84	TF 0.84%	4
Burton[34]	Saloum, Gambia	Longitudinal	>6 months	14 villages	TF 8% *Ct* infection 7%	83%	Single	0, 2, 6, 12, 17	TF 4% *Ct* infection 3%	2
Harding–Esch[35]	Gambia and Senegal	RCT Standard *vs*. Enhanced cove rage Single *vs*. three rounds	<5 years	48 communities	TF 6.50%; *Ct* infection: 0.80%	80–90 or >90	Single or Annual for 3 years	36	TF/*Ct* infection: Standard (2.30%/1.00%) *vs*. Enhanced coverage (3.00%/0.20%); Single round (2.50%/0.20%) *vs*. 3 round (2.30%/1.00%); All	1

									$P > 0.050$	
Kalua[36]	Malawi	Cross sectional	1–9 years	432 clusters	TF 5.00–9.90%	80	Single	at least 6	TF 0.00–2.90%	4
Wilson[37]	Kongwa, Tanzania	Longitudinal	1–9 years	96 sub-village units	TF 5.00–9.90%	73	Single	0, 12	Treated: TF 9.30% Non-treated: TF 8.20%	2
Annual MDA for 2 years										
High baseline prevalence										
Cumberland[38]	Gurage and Oromia, Ethiopia	Cross sectional	3–9 years	40 communities	TF 71.40%; *Ct* infection: 12.00%	92	Annual, 1–3 years	36	TF 35.60%; *Ct* infection 3.10% 2 or 3 doses *vs.* 1 dose: OR = 0.30 (95% CI 0.10–0.91)	4
Astle[39]	Southern Zambia	Cross sectional	1–10 years	Gwembe district	TF 55%	NR	Annual, 2 years	24	TF 10.60%	4
West[40]	Kongwa, Tanzania	Longitudinal	<15 years	1 village	TF 31.20–53.30%; *Ct* infection: 56.80–70.70%	>80	Annual, 2 years	18, 60	TF 5.60–33.80%; *Ct* infection 16.90–28.20%	2
Roba[41]	Four regions of Ethiopia	Cross sectional	1–9 years	8 districts in four provinces	TF 36.70%	59	Annual, 1–3 years	36	TF 18.40%	4
Moderate baseline prevalence										
Ngondi[42]	Southern Sudan	Cross sectional	1–9 years	25 villages	TF 23.10%	53	Annual, 1–3 years	60	TF: 19–27%	4
Nash[43]	Amhara, Ethiopia	Cross sectional	1–9 years	4 districts	Andabet: TF 30.60%; Dera: TF	>80	Annual, 1–3 years	at 2017	Andabet: TF 37.00%; antibodies 36.90% Dera: TF 14.70%;	4

					29.10% Woreta town: TF 5.40% Alefa: TF 4.60% (2011–2016) Upper West: TF 15.00% Northern Region: TF 9.10% (2000)				antibodies 11.30% Woreta town and Alefa: TF <5%; antibodies <5% *Ct* infection was only detected in Andabet (11.30%)	
Hagan[44]	Northern and Upper West Regions, Ghana	Cross sectional	1–10 years	2 districts in Upper West and 3 in Northern Regions		>90	Annual, 2 years	At 2003	Upper West: TF 5.60% Northern Region: TF 3.50%	4
Low baseline prevalence										
Sanders[9]	Sudan	Cross sectional	1–9 years	7 localities	6 localities: TF 5.90–8.50% 1 locality: TF 17.40%	>80	Annual, 1–3 years	36	TF 0.40–6.40%; 4/7 localities achieved TF <5%	4
Amnie[45]	Plateau and Nasarawa States of Nigeria	Cross sectional	1–9 years	793 households	TF 6.40%	58.20–60.30	Annual, 2 years	24	TF 3.40%	4
Annual MDA for 3 or more years										
High baseline prevalence										
Keenan[46]	Ethiopia	Cross sectional	<9 years	12 subkebeles	TF 69%; *Ct* infection: 42%	80.90–92.10	Annual, 3 years	36	TF 43.50%; *Ct* infection: 4.20%	4
Ayele[47]	Amhara, Ethiopia	Cross sectional	<10 years	12 communities	TF 68%	>80	Annual, 3 years	12, after the third MDA	TF: 43.30%	4
Ngondi[48]	Southern Sudan	Cross sectional	1–9 years	4 sites	Kiech Kuon: TF 61%	Kiech Kuon:	Annual, 3 years	36	TF: Kiech Kuon: 60%; Padak: 42%; Katigiri: 4%;	4

					Padak: TF 58% Katigiri, TF 48% Tali: TF 67%	14%; others: 69–75			Tali: 6%	
Ngondi[49]	Amhara, Ethiopia	Cross sectional	1–9 years	5 districts	TF 49–90%	70–90	Annual, 3 years	0, 36	TF 11–54% *Ct* 3.1%	4
West[50]	Kongwa, Tanzania	Cross sectional	<5 years	71 communities	TF 51%	<78	Annual, 3–7 years	96	3 rounds: TF 20.30% 4 rounds: TF 9.30% 5 rounds: TF 12.80% 6 rounds: TF 12.60% 7 rounds: TF 9.10%	4
Malhotra[51]	Car-Nicobar Island, India	Cross sectional	1–9 years	15 villages	TF 50.50%	>80	Annual, 3 years	12, after the last MDA	TF 6.80%	4
Reda[52]	Tigray, Ethiopia	Cross sectional	1–9 years	Deguatemben district	TF 44.70%	>90	Annual, 3 years	36	TF 18.70%	4
Admassu[53]	Gurage, Ethiopia	Cross sectional	1–9 years	1 district	TF >40% (estimated)	>90	Annual, 3 years	60	TF 22.80%	4
Nash[54]	Amhara, Ethiopia	Longitudinal	1–5 years	58 districts	TF >30% (estimated)	NR	Annual, 5 years	60	TF: 39.90% *Ct* infection: 6.00%	2
Nash[55]	Amhara, Ethiopia,	Longitudinal	1–5 years	All districts of Amhara	TF >30 (estimated)	NR	Annual, 5 years	60	TF: 26.20% *Ct* infection: 5.70%	2
Bamani[56] (first survey)	Kayes and Koulikoro, Mali	Cross sectional	1–9 years	16 districts	TF 33.5–42.50%	43.90–86.10	Annual, 3 years	6, 36, after last MDA	6 months: Kayes: TF 3.90%; Koulikoro: TF 2.70% 3 years: Kayes: TF 7.26%; Koulikoro: TF 8.19%	4
Traoré[57] (second survey)	Kayes, Mali	Cross sectional	1–9 years	4 districts	TF 33.50–42.50%	43.90–86.10	Annual, 3 years	96, after stopping MDA	TF 0.20–0.80% (Intensified effort of "S"+"F"+"E" was taken after the first survey)	4

Sanders[58]	South Sudan	Cross-sectional	1–9 years	5 countries	TF >30%	Low	Annual, ≥4 years	>2 years after the last MDA	TF: Budi 17.40% Lafon 35.30% Kapoeta East: 47.60% Kapoeta North: 39.70% Kapoeta South: 30.10%	4
Stewart[59] (first survey)	Amhara, Ethiopia	Longitudinal	1–9 years	152 districts	TF 32.70%	>90	Annual, 3–5 years	During 2010–2015	TF: 25.90%; TF <5% (6%): 9 districts TF 5.00–9.90% (11%): 17 districts TF 10.00–29.90% (46%): 70 districts TF ≥30% (37%): 56 districts	2
Sata[8] (second survey)	Amhara, Ethiopia	Longitudinal	1–9 years	160 districts	TF: see first survey	90	Annual, continued 4 years (7–9 years)	During 2015–2019	TF <5% (28%): 45 districts TF ≥30% (19%): 30 districts Reached TF< 5% at the second survey: 20/26 districts with TF <10% at the first survey; 21/75 districts with TF 10.00–29.90% at the first survey; 4/59 districts with ≥30% TF at the first survey.	2
Mpyet[60]	Kebbi, Sokoto and Zamfara States, Nigeria	Cross sectional	1–9 years	15 LGAs	TF 13.50–49.00% (12 LGAs <30%; 3 LGAs 30–50%) (2004–2006)	NR	Annual, 3–5 years (3 years: 12 LGAs; 5 years: 3 LGAs)	During 2014–2016	TF <5%: 8/15 (53%) LGAs TF 5.00–9.90%: 6/15 (40%) LGAs TF ≥10%: 1/15 (6%) LGAs	4

Lee[61]	Kongwa, Tanzania	Cross sectional	<5 years	32 communities	TF 31%; *Ct* infection: 22%	>80	Annual, 3 years	0, 12, 24, 36	TF 8%; *Ct* infection:5%	4
West[64]	Kongwa, Tanzania	RCT Standard *vs*. Enhanced cove rage	<5 years	32 communities	TF >30%; *Ct* infection: about 20%	80–90 or >90	Annual, 3 years	Every 6 for 36	TF/*Ct* infection: Standard (6.10%/4.00%) *vs*. Enhanced (9.00%/5.40%); All $P > 0.050$	1
Moderate baseline prevalence										
Yohannan[62]	Kongwa, Tanzania	Longitudinal	<10 years	4 villages	TF 27.70%; *Ct* infection: 23.60%	80	Annual, 4 years	Every 6 for 42	TF 7.90%; *Ct* infection: 5.10%	2
Shekhawat[63]	Kongwa, Tanzania	Longitudinal	<10 years	3 communities	TF 27.30%	>85	Annual, 3 years	0, 12, 24, 48	TF 9.40%	2
Amza[65]	Zinder, Niger	RCT Standard *vs*. Enhanced coverage	<5 years	24 communities	TF 20–30%; *Ct* infection: about 20%	80–90 or >90	Annual, 4 years	Every 6 for 36	TF/*Ct* infection: Standard (7.10%/4.60%) *vs*. Enhanced (8.90%/7.10%); All $P > 0.050$	1
Pant[5]	Achham, Nepal	Cross sectional	1–9 years	24 communities	TF 20%	Unknown	Annual, 4 years	72	TF 0.30%; *Ct* infection: 0%	4
Burr[66]	Chikwawa and Mchinji, Malawi	Cross sectional	1–9 years	6 EUs	TF 13.60–21.70%	NR	Annual, 3 years	12,after last MDA	TF 4.70–7.20% *Ct* infection 0.10–0.70% Pgp3 seroprevalence 6.90–12.00% Two EUs have TF <5%	4
Yohannan[67]	Kongwa, Tanzania	RCT 3 rounds *vs*. cessation rule (also 3 rounds)	<5 years	16 communities	TF 12–13%; *Ct* infection: 5.40–6.40%	82.70–94. 20	Annual, 3 years	0, 6, 12, 18	TF 7–9%; *Ct* infection: 3 rounds (2.90%) *vs*. cessation rule (4.70%); $P = 0.250$	1
Hiep[68]	Hanoi,	Cross sectional	1–9 years	11 EUs	7 EUs have	NR	Annual, >3	At 2014	All EUs: TF 0.00–1.60%	4

	Viet Nam				TF >10% 4 EUs have TF <10% (2003–2011)		years			
Biannual										
High baseline prevalence										
Lakew[69]	Gurage, Ethiopia	Longitudinal	1–5 years	16 communities	TF 91.60%; *Ct* infection: 63.50%	94.10	Biannual, 2 years	Every 6 for 42	TF NR; *Ct* infection: 6 months after the last MDA: 2.60% 2 years after the last MDA 25.20%	2
Ewald[70]	Aboriginal community in Australia	Cross-sectional	1–9 years	1 community	TF 40%	Low	at baseline, 7 months and 21 months	0, 7, 21	TF: 33% at 7 months and 37% at 21 months	4
Biebesheimer[71]	Gurage, Ethiopia	Cross sectional	1–5 years	2 villages	TF 78–83%; *Ct* infection: 48%	>90	Biannual, 3 years	Every 6 for 42	TF 17% and 24% in the two villages; *Ct* infection: 0%	4
Moderate baseline prevalence										
Oldenburg[72]	Matameye, Niger	RCT Standard *vs*. Enhanced coverage	<5 years	24 communities	TF 20–30%; *Ct* infection: 15–25%	80–90 or >90	Biannual, 3 years	36	TF/*Ct* infection: Standard (10.10%/4.40%) *vs*. Enhanced (5.40%/3.30%); $P > 0.200$/$P = 0.600$	1
Different frequencies of MDA										
High baseline prevalence										

Gebre[73] (TANA I)	Amhara, Ethiopia	RCT Annual *vs*. Biannual	<9 years	24 subdistricts	TF: 68–84%; *Ct* infection: 38–42%	>80	Annual or Biannual, 4 years	Every 6 for 42	TF/*Ct* infection: Annual (31.50%/1.90%) *vs.* Biannual (35%/3.20%); *P* = 0.120/*P* > 0.990	1
Keenan[74] (TANA II)	Amhara, Ethiopia	RCT Annual *vs*. Biannual;	<9 years	48 communities	See TANA I at follow up	>90	Annual or Biannual, continued 3 years (7 years)	54, 66 , 78, 90	TF/*Ct* infection: Annual (39.80%/9.90%) *vs*. Biannual (25.40%/3.30%): *P* > 0.050/*P* = 0.090	1
Melese[75]	Gurage, Ethiopia	RCT Annual *vs*. Biannual	1–5 years	16 rural villages	TF >70%; *Ct* infection: 30–43%	90.80	Annual or Biannual, 2 years	0, 2, 6, 12, 18, 24	*Ct* infection: Annual (6.80%) *vs.* Biannual (0.90%); *P* = 0.001	1
House[76]	Amhara, Ethiopia	RCT Annual *vs*. Quarterly for 1–10 years children	1–10 years	36 subkebeles	TF 69%; *Ct* infection: 40–50%	76–88	Annual or Quarterly,1 years	12	*Ct* infection: Annual (11.60%) *vs*. Quarterly (3.60%); *P* = 0.001	1
Lietman[77]	Amhara, Ethiopia	RCT Annual *vs*. Biennial/ Biannual/Quarterly for 1–10 years children)	1–9 years	60 communities	TF 54–73%; *Ct* infection: 37.00–51.80%		Annual，Biannual, or Quarterly, 1 years	12, 24	*Ct* infection: 12 months: Delayed (46.30%) *vs*. Annual (14.50%) *vs*. Biannual (4.70%) *vs*. Quarterly (1.90%) 24 months: Biennial (14.20%) *vs*. Annual (8.00%) *vs*. Biannual (4.50%):	1

Moderate baseline prevalence

Amza[78]	Matameye, Niger	RCT Annual *vs*. Biannual for 6 months–12 years children)	<5 years	48 communities	TF 20–30%; *Ct* infection: about 20%	>80	Annual or Biannual, 3 years	Every 6 for 36	TF/*Ct* infection: Annual (8.00%/5.80%) *vs.* Biannual (7.8/3.80%); $P = 0.670$/$P > 0.050$	1

*Prevalence of disease at the last follow-up time.†Quality rating was based on Oxford Centre for Evidence-based Medicine's Levels of Evidence and Grades of Recommendation. CI: Confidence interval; *Ct*: *Chlamydia trachomatis*; EU: Evaluation units; LGA: Local government areas; MDA: Mass drug administration; NR: Not reported; OR: Odds ratio; RCT: Randomized controlled trials; TF: Trachomatous inflammation-follicular.

Supplementary Table 2: Risk of bias of studies on azithromycin MDA for trachoma*.

Study	Confounding		Selection bias		Misclassification bias	Performance bias		Detection bias	Attrition bias	Reporting bias
	Bias due to confounding (O)	Selection into study (O)	Random sequence generation (R)	Allocation concealment (R)	Bias in intervention classification(O)	Bias due to deviations from interventions (O)	Masking of participants and personnel (R)	Masking of outcome assessors (R, O)	Incomplete outcome data (R, O)	Selective reporting (R, O)
Cluster-RCT										
Amza 2017[78]	NA	NA	Low	Unclear	NA	NA	Unclear	Low	Low	Low
Amza 2018[65]	NA	NA	Low	Unclear	NA	NA	Unclear	Unclear	Low	Low
Gebre 2012[73] (TANAI)	NA	NA	Low	Low	NA	NA	Low	Low	Low	Low
Keenan 2018[74] (TANA II)	NA	NA	Low	Low	NA	NA	High	Low	Low	Low
Harding–Esch 2013[35]	NA	NA	Moderate	Unclear	NA	NA	Moderate	Low	Low	Low
Holm 2001[31]	NA	NA	Moderate	Unclear	NA	NA	Unclear	Low	Low	Low
House 2009[76]	NA	NA	Low	Unclear	NA	NA	Unclear	Low	Low	Low

Lietman 2020[77]	NA	NA	Unclear	Unclear	NA	NA	High	Low	Low	Low
Melese 2008[75]	NA	NA	Moderate	Unclear	NA	NA	Unclear	Low	Low	Low
Oldenburg 2018[72]	NA	NA	Low	Unclear	NA	NA	Unclear	Low	Low	Low
West 2006[18]	NA	NA	Low	Unclear	NA	NA	Unclear	Low	Low	Low
West 2013[64]	NA	NA	Low	Low	NA	NA	Low	Low	Low	Low
Yohannan 2013[67]	NA	NA	Low	Unclear	NA	NA	Unclear	Low	Low	Low
Non-RCT										
Admassu 2013[53]	Low	Low	NA	NA	Low	Moderate	NA	Unclear	Low	Low
Amnie 2018[45]	Low	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Astle 2006[39]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Ayele 2011[47]	Unclear	Unclear	NA	NA	Low	Low	NA	Unclear	Low	Low
Bamani 2010[56]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Biebesheimer 2009[71]	Unclear	Low	NA	NA	Unclear	Unclear	NA	Unclear	Unclear	Low
Broman 2006[16]	Unclear	Low	NA	NA	Low	Unclear	NA	Unclear	Low	Low
Burr 2019[66]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Burton 2010[32]	Moderate	Low	NA	NA	Low	Moderate	NA	Unclear	Low	Low
Burton 2005[34]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Cajas-Monson 2011[26]	Low	Unclear	NA	NA	Low	Low	NA	Unclear	Low	Low
Chidambaram 2006[22]	Unclear	Low	NA	NA	Low	Unclear	NA	Unclear	Low	Low
Chidambaram 2004[25]	High	Moderate	NA	NA	Moderate	Low	NA	Unclear	Moderate	Moderate
Cumberland	High	Low	NA	NA	Low	Moderate	NA	Unclear	Low	Low

2008[38]										
Ewald 2003[70]	Unclear	Unclear	NA	NA	Low	Low	NA	Unclear	Low	Low
Hagan 2009[44]	Unclear	Low	NA	NA	Low	Low	NA	Moderate	Low	Low
Harding-Esch 2019[29]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Hiep 2018[68]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Kalua 2018[36]	High	Low	NA	NA	Low	Unclear	NA	Unclear	Moderate	Low
Keenan 2012[46]	Moderate	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Lakew 2009[69]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Lakew 2009[21]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Laming 2000[19]	Unclear	Unclear	NA	NA	Low	Low	NA	Unclear	Low	Low
Last 2017[28]	Moderate	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Lee 2014[61]	Moderate	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Malhotra 2016[51]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Melese 2004[2]	Low	Low	NA	NA	Low	Low	NA	Low	Low	Low
Morberg 2018[20]	Low	Low	NA	NA	Low	Unclear	NA	Unclear	Low	Low
Mpyet 2018[60]	Unclear	Low	NA	NA	Unclear	Unclear	NA	Unclear	Low	Low
Nash 2021[43]	Unclear	Low	NA	NA	Low	Moderate	NA	Low	Low	Low
Nash 2020[54]	Low	Low	NA	NA	Unclear	Unclear	NA	Unclear	Low	Low
Nash 2018[55]	Low	Low	NA	NA	Unclear	Unclear	NA	Unclear	Low	Low
Ngondi 2009[49]	Low	Low	NA	NA	Low	Moderate	NA	Unclear	Low	Low
Ngondi 2008[42]	Moderate	Low	NA	NA	Low	Moderate	NA	Unclear	Low	Low
Ngondi 2006[48]	Low	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Pant 2016[4]	Unclear	Low	NA	NA	Low	Unclear	NA	Unclear	Low	Low

Reda 2020[52]	Unclear	Moderate	NA	NA	Unclear	Unclear	NA	Unclear	Low	Low
Roba 2011[41]	Unclear	Low	NA	NA	Low	Low	NA	Moderate	Low	Low
Sanders 2019[9]	Low	Low	NA	NA	Moderate	Moderate	NA	Unclear	Low	Low
Sanders 2017[58]	Low	Low	NA	NA	Moderate	Moderate	NA	Unclear	Low	Low
Sata 2021[8]	Low	Low	NA	NA	Unclear	Unclear	NA	Unclear	Moderate	Low
Schémann 2007[27]	Unclear	Low	NA	NA	Low	Moderate	NA	Unclear	Low	Low
Shekhawat 2014[63]	Low	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Solomon 2008[24]	Unclear	Unclear	NA	NA	Unclear	Low	NA	Unclear	Low	Low
Solomon 2004[23]	Low	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Stewart 2019[59]	Low	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Traoré 2018[57]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
West 2011[50]	Unclear	Low	NA	NA	Moderate	Moderate	NA	Unclear	Low	Low
West 2011[30]	Unclear	Low	NA	NA	Low	Unclear	NA	Unclear	Low	Low
West 2005[17]	Moderate	High	NA	NA	Low	Unclear	NA	Unclear	Low	Low
West 2007[40]	Unclear	Low	NA	NA	Moderate	Moderate	NA	Unclear	Low	Low
Wilson 2019[37]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low
Yayemain 2009[33]	Moderate	Low	NA	NA	Low	Low	NA	Moderate	Low	Low
Yohannan 2014[62]	Unclear	Low	NA	NA	Low	Low	NA	Unclear	Low	Low

*Assessed using the Cochrane Collaboration's risk of bias tool for randomized interventions and Cochrane's ROBINS-I tool for non-randomized interventions. O=domain defined for observational, non-randomized studies; R=domain defined for randomized studies.